

Hemostatic powders as monotherapy or a rescue therapy in malignancy-related gastrointestinal bleeding: urgent need for large randomized controlled trial



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To the Editor:

We read with interest the article by Karna et al¹ on the efficacy of topical hemostatic agents, mainly hemostatic powder (HP), in malignancy-related GI bleeding (GIB). This condition is an endoscopic challenge, and a criterion standard hemostatic technique remains to be defined. The metanalysis, including 16 studies with 530 patients, revealed a primary hemostasis rate of 94.1%, with an aggregate (early and delayed) rebleeding rate of 24.2%. A limitation of the study lies in the scarcity of published data, as stated by the authors themselves. Despite an extensive evaluation of >350 citations, the authors could select only 2 randomized controlled trials—a pilot including 10 patients² and a preliminary abstract on 18 patients³—and 14 studies with different methodologies and heterogeneous lesions. The disease stage affects the outcome; it is hard to achieve stable hemostasis in diffuse lesions with multiple bleeding points. HP is a straightforward procedure, well suited for malignancy-related GIB because of the possibility of spreading the hemostatic agent over a large area of bleeding. A key point is whether it is preferable to use HP as a first choice or rescue therapy, namely, as an alternative to, or after the failure of, other hemostatic procedures. In our experience, HP used as a first choice or rescue therapy achieved immediate hemostasis in 100% and 85.7% but definitive hemostasis (no rebleeding) in 45.5% and 85.7%, respectively, of patients with malignancy-related GIB.⁴ Taken together, the available data preclude drawing a definitive conclusion as to whether HP is an alternative to other hemostatic methods in malignancy-related GIB. Prospective large multicenter studies are urgently needed to define the appropriate use of HP, as a standalone or rescue therapy, and whether it may be considered the criterion standard in malignancy-related GIB.

DISCLOSURE

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Response:



We thank Paoluzi et al¹ for their encouraging letter in response to our recently published meta-analysis on the efficacy of topical hemostatic agents in malignancy-related GI bleeding (GIB).² Despite our literature search efforts, only 2 randomized controlled trials were available for inclusion, and we were unable to answer the key question of whether topical Hemospray (Cook Medical, Bloomington, Ind, USA) could be a preferable first-choice therapy or rescue therapy. We agree with Paoluzi et al¹ that prospective large multicenter studies are needed to define the utility of hemostatic agents as a first-line or rescue agent in comparison with other hemostatic methods.

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